

Policy on Cardio-pulmonary Resuscitation Services

«Hospital Name»

Document control

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Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

Cardio-pulmonary resuscitation services shall be available and provided to patients at all times. «Hospital Name» shall maintain a documented CPR procedure, appropriate equipment and medications, trained team roles, event recording, post-event analysis and corrective and preventive measures.

1. Purpose

This policy establishes requirements for providing cardio-pulmonary resuscitation services, maintaining CPR readiness, recording CPR events and mock drills, analysing outcomes, and implementing corrective and preventive measures.

2. Scope

This policy applies across all areas of «Hospital Name» where cardio-pulmonary resuscitation may be required, including the patient care areas identified in this policy.

3. Policy standards

3.1 CPR procedure and immediate response

The organisation shall document the procedure for cardio-pulmonary resuscitation (CPR) for adults across all areas in the organisation. This shall be in consonance with accepted practices.

Where appropriate, it shall also address obstetric, paediatric and neonatal patients.

The organisation shall ensure that medical equipment for resuscitation and medications for basic and advanced life support are provided in standardised manner.

Basic life support shall be initiated as soon as a condition requiring CPR is identified. This shall be implemented in all areas of the organisation.

The protocols could be displayed prominently in all critical areas such as emergency, ICU, OT and all crash carts.

3.2 CPR team roles and responsibilities

CPR team members shall have a clear understanding of their roles and responsibilities during resuscitation and shall comply with their assigned roles and responsibilities to effectively function as a team.

3.3 Equipment and medications

At a minimum, emergency medications and equipment for intubation based on the needs of the patients served shall be available in patient care areas including the blood centre, radiology, day care, dialysis, chemo ward, OPD, rehabilitation services areas, endoscopy, and in areas where any invasive procedure is performed.

Other equipment like defibrillator shall be easily accessible to ensure that there is no delay in cardio-pulmonary resuscitation.

It is preferable that the minimum emergency medication is standardised across the organisation.

3.4 Recording CPR events and mock drills

In the actual event of cardio-pulmonary resuscitation, or a mock drill of the same, all the activities along with the personnel attended shall be recorded.

At the minimum, it will include timeliness of response, availability of human resources, equipment, drugs, and barriers if any.

The recording could be done using the pre-defined procedural checklist and by monitoring whether the prescribed activity has been performed properly and in the right sequence.

It is a good practice to debrief team members regarding the necessary immediate corrective and preventive action.

3.5 Post-event analysis

The frequency of the committee meeting shall be at least once a quarter.

The analysis shall focus on the initiation of CPR, time of arrival of the team, availability of required resources, recording of the sequence of events during CPR (including technique) and the overall coordination.

The organisation shall monitor outcome of CPR and identify areas for improvement.

The multidisciplinary committee shall be independent and include at least one physician/cardiologist, one anaesthesiologist, one member from the code blue team and one nurse.

The analysis shall be completed within a defined time frame, as determined by the Multidisciplinary CPR Committee and recorded in the CPR Event Review procedure.

3.6 Corrective and preventive measures

Corrective and preventive measures shall be completed within a defined time frame, as determined by the Multidisciplinary CPR Committee and recorded in the CPR Event Review procedure.

The findings of the post-event analysis shall be communicated to the personnel who participated in the CPR.

Any lapses shall be discussed, with the view to improve the outcomes in future.

During subsequent resuscitations, it is preferable that implementation of these actions is noted and training be modified, if necessary.

4. Non-negotiable rules

- Do not start CPR without initiating basic life support as soon as a condition requiring CPR is identified.
- Do not start a planned resuscitation response without a named CPR team with clear roles and without the minimum emergency medications and equipment available at the location.
- Do not use a CPR process without the documented CPR procedure and standardised resuscitation equipment and medications required by this policy.
- Do not omit emergency medications and intubation equipment from the specified patient care areas.
- Do not conduct CPR where the defibrillator cannot be accessed without delay.
- Do not omit recording of an actual CPR event or mock drill, including timeliness of response, availability of human resources, equipment, drugs, and barriers if any.
- Do not complete a post-event analysis without the multidisciplinary committee meeting at least once a quarter and analysing the specified CPR performance areas.
- Do not omit the independent multidisciplinary committee composition specified in this policy.
- Do not leave corrective and preventive measures incomplete beyond the defined time frame.
- Do not withhold post-event analysis findings from personnel who participated in the CPR.

5. What we do

5.1 CPR procedure and readiness

- Maintain a documented adult CPR procedure across all areas in consonance with accepted practices.
- Where appropriate, address obstetric, paediatric and neonatal patients.
- Provide resuscitation equipment and basic and advanced life-support medications in a standardised manner.
- Initiate basic life support as soon as a condition requiring CPR is identified in all areas.
- Protocols could be displayed prominently in critical areas such as emergency, ICU, OT and crash carts.

5.2 CPR team

- Ensure CPR team members understand and comply with their assigned roles and responsibilities.

5.3 Equipment and medication coverage

- Keep at least the required emergency medications and intubation equipment based on patient needs in the blood centre, radiology, day care, dialysis, chemo ward, OPD, rehabilitation services areas, endoscopy, and areas where an invasive procedure is performed.
- Keep the defibrillator easily accessible so there is no delay in CPR.
- Standardisation of the minimum emergency medication is preferable.

5.4 Event recording

- Record all activities and personnel involved in actual CPR events and mock drills.
- Record timeliness of response, availability of human resources, equipment, drugs, and barriers if any.
- A predefined procedural checklist could be used.
- Debriefing team members regarding immediate corrective and preventive action is good practice.

5.5 Post-event analysis

- Have the multidisciplinary committee meet at least once a quarter.
- Analyse initiation of CPR, time of arrival of the team, availability of required resources, recording of the sequence of events during CPR including technique, and overall coordination.
- Monitor CPR outcomes and identify areas for improvement.
- Maintain an independent multidisciplinary committee with at least one physician/cardiologist, one anaesthesiologist, one member from the code blue team and one nurse.
- Complete the analysis within a defined time frame.

5.6 Corrective and preventive measures

- Complete corrective and preventive measures within a defined time frame.
- Communicate post-event analysis findings to personnel who participated in the CPR.
- Discuss lapses to improve outcomes in future.
- Preferably note implementation of actions during subsequent resuscitations and modify training if necessary.

6. Stop-work authority

Do not start a planned resuscitation response (or mock drill counted as CPR competence evidence) without a named CPR team with clear roles and without the minimum emergency medications and equipment available at the location. Stop-work does not block an unexpected cardiac arrest already in progress — start CPR with available staff and escalate for equipment/team immediately. The person responsible tells the CPR Committee chair or Emergency In-Charge the same shift. Refusing to run a hollow CPR response is not a disciplinary matter.

The post-event analysis committee is a separate governance and quality loop, not part of the stop-work trigger. Stop-work concerns CPR readiness at the moment of need, not analysis-timeline compliance.

7. Governance and responsibility

Role	Responsibility
CPR team members	CPR team members are responsible for understanding and complying with their assigned roles and responsibilities during resuscitation.
Multidisciplinary CPR committee	Responsible for post-event analysis, CPR outcome monitoring and identification of areas for improvement. The committee shall be independent and include at least one physician/cardiologist, one anaesthesiologist, one member from the code blue team and one nurse.

8. Quality monitoring

Monitoring area	What is monitored
CPR event recording	CPR events and mock drills, including the required minimum record content
Committee meetings	Committee meetings at least once a quarter
Analysis coverage	Analysis of initiation of CPR, time of arrival of the team, availability of required resources, recording of the sequence of events during CPR including technique, and

	overall coordination
CPR outcomes	CPR outcomes and areas for improvement
CAPA completion	Completion of corrective and preventive measures within the defined time frame
Post-event communication	Communication of post-event findings to CPR participants

9. Training and staff acknowledgement

Training and mock drills form part of the documented CPR arrangements. CPR team members shall understand their assigned roles and responsibilities.

I have read the Policy on Cardio-pulmonary Resuscitation Services of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

The policy shall be available to personnel involved in CPR services, including CPR team members and personnel working in patient care areas.

11. Abbreviations

Abbreviation	Meaning
BLS	Basic Life Support
ALS	Advanced Life Support
CPR	Cardio-pulmonary Resuscitation
CAPA	Corrective and Preventive Action
ICU	Intensive Care Unit
OT	Operation Theatre
OPD	Outpatient Department

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.5.a	Commitment	Sections 3.1 and 5.1 address the documented CPR procedure for adults across all areas, standardised resuscitation equipment

		and medications, and immediate BLS initiation. Stop-work authority (Section 6) also applies to this element.
COP.5.b	Commitment	Sections 3.2 and 5.2 address CPR team compliance with assigned roles and responsibilities. Stop-work authority (Section 6) also applies to this element.
COP.5.c	Commitment	Sections 3.3 and 5.3 address emergency medications and intubation equipment in the nine specified areas and areas where invasive procedures are performed, and defibrillator accessibility. Stop-work authority (Section 6) also applies to this element.
COP.5.d	Commitment	Sections 3.4 and 5.4 address recording of actual CPR events and mock drills, including the five minimum content items.
COP.5.e	Commitment	Sections 3.5 and 5.5 address post-event analysis: quarterly committee meetings, five analysis areas, CPR outcome monitoring, independent committee with minimum composition and analysis within a defined time frame.
COP.5.f	Commitment	Sections 3.6 and 5.6 address CAPA within a defined time frame, findings communicated to CPR participants, and lapses discussed for future improvement.

13. Required Records/Evidence Checklist

CPR procedure and readiness

- Documented adult CPR procedure across all areas in consonance with accepted practices.

- Written provisions, where appropriate, for obstetric, paediatric and neonatal CPR.
- Standardised resuscitation equipment and BLS/ALS medications across the organisation.
- Evidence that BLS is initiated immediately on identification of a CPR-requiring condition in all areas.

Equipment and medication coverage

- Emergency medications and intubation equipment available in the blood centre, radiology, day care, dialysis, chemo ward, OPD, rehabilitation services areas, endoscopy, and all areas where invasive procedures are performed.
- Defibrillator accessible without delay in CPR locations.

CPR event and mock drill recording

- Records of actual CPR events and mock drills including personnel attended.
- Minimum record content: timeliness of response, availability of human resources, equipment, drugs, and barriers if any.

Post-event analysis

- Evidence that the multidisciplinary committee meets at least once a quarter.
- Analysis records covering: initiation of CPR, time of arrival of the team, availability of required resources, recording of event sequence including technique, and overall coordination.
- CPR outcome records and identified areas for improvement.
- Evidence of independent multidisciplinary committee with at least one physician/cardiologist, one anaesthesiologist, one member from the code blue team and one nurse.
- Evidence of analysis completed within the defined time frame.

CAPA and communication

- CAPA records showing measures completed within the defined time frame.
- Evidence that post-event analysis findings were communicated to CPR participants.
- Records of lapses discussed and actions taken for future improvement.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. COP.5.

Disclaimer

This policy reorganises the supplied COP.5 objective elements and Guidebook interpretation into plain language. Mandatory requirements and their stated modal

strength have been retained. Illustrative examples, optional approaches and references have not been converted into mandatory requirements.